Modified FOLFOX-6 + bevacizumab

National Cancer Control Programme (NCCP) 00446 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first treatment

Confirm the aim, likely course length, blood-test arrangements and a working urgent contact number. dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing is required before starting the fluoropyrimidine unless already completed.

STEP 2

Line and pump arrangements

Confirm venous access, the 46-hour fluorouracil pump and disconnection arrangements before leaving. Know whom to contact for leakage, an alarm, line pain or a pump that finishes unexpectedly.

STEP 3

If treatment is delayed

Ask the unit to confirm the new treatment and blood-test dates. Re-anchor supportive medicines to the treatment actually given; do not take an extra or catch-up anticancer dose yourself.

STEP 4

Getting help between visits

Follow your oncology unit's urgent advice for fever, infection, severe diarrhoea, chest pain or difficulty breathing. Bring your treatment card and current medicines list.

STEP 5

Operations and dental procedures

Tell both the oncology team and the procedural team that you receive bevacizumab. Holds and restart need an agreed wound-healing plan; do not stop or restart it yourself.

STEP 6

Paper copies, wallet QR and contacts

The example telephone numbers on the wallet card are placeholders. Your team must replace them with working daytime and out-of-hours numbers before patient use. Keep the card with you; the QR opens the regimen information, not your confidential medical record.

Sources: NCCP 00446 v7b · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.